

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: Based on a medium preeclampsia risk tier, the patient at 32 weeks gestation presents with hypertension (systolic BP 142 mmHg) and proteinuria (dipstick 1). No severe preeclampsia symptoms are reported. The macrosomia risk tier is also medium, with a high pre-pregnancy BMI (42.4) and glucose levels within generally healthy ranges. An alert to the doctor is recommended for further evaluation of the preeclampsia risk.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient TEST_MED_MED, 32 weeks gestation, presents with a medium preeclampsia risk tier. Current vitals: systolic BP 142 mmHg, diastolic BP 79 mmHg, proteinuria dipstick 1. No severe symptoms (headache, vision changes, abdominal pain, facial/hand swelling) reported. This patient's systolic BP meets the definition of hypertension in pregnancy (≥140/90 mmHg) per ACOG guidelines. The macrosomia risk tier is also medium, with a pre-pregnancy BMI of 42.4. Further clinical evaluation for preeclampsia is recommended.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

⚠ This report is generated by an AI decision-support system and does not replace clinical judgment. Always follow institutional protocols.